IN THE SUPREME COURT OF IOWA
SUPREME COURT NO: 25-1433

MICHAEL WOOFF
Petitioner-Appellant,

vs.

NINE PARK LANE, INC. d/b/a MENEFEE DRYWALL and WESTERN NATIONAL INSURANCE COMPANY
Respondents-Appellees

APPEAL FROM THE IOWA DISTRICT COURT
IN AND FOR LINN COUNTY
Case No: CVCV106242
THE HONORABLE CHAD KEPROS

APPELLANT'S REPLY BRIEF

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STATEMENT OF THE ISSUE PRESENTED FOR REVIEW

A. WHETHER THE COMMISSIONER COMMITTED AN ERROR OF LAW IN CONCLUDING THAT DEFENDANTS OFFERING WOOFF JOBS THAT HE WAS UNQUALIFIED TO DO WAS SUFFICIENT TO AVOID AN INDUSTRIAL DISABILITY AWARD

B. WHETHER SUBSTANTIAL EVIDENCE SUPPORTS THAT WOOFF'S MENTAL HEALTH INJURY SHOULD BE CONSIDERED IN THE INDUSTRIAL DISABILITY ANALYSIS

C. WHETHER THE COMMISSIONER ERRED AS A MATTER OF LAW WHEN IT IMPROPERLY USED AGENCY EXPERTISE IN DETERMINING WOOFF'S PERMANENT IMPAIRMENT

ARGUMENT

STANDARD OF REVIEW

The Iowa Supreme Court has discussed the standard of review for different appellate grounds in the workers compensation system as follows:

> If the claim of error lies with the agency's findings of fact, the proper question on review is whether substantial evidence supports those findings of fact. If…the claim of error lies with the ultimate conclusion reached, then the challenge is to the agency's application of the law to the facts, and the question on review is whether the agency abused its discretion by, for example, employing wholly irrational reasoning or ignoring important and relevant evidence. See, Iowa Code § 17A.19(10)(i), (j). *Meyer v. IBP, Inc.*, 710 N.W.2d 213, 218 (Iowa 2006) (emphasis added).

The standard of judicial review of an agency decision is governed by Iowa Code chapter 17A, Iowa's Administrative Procedure Act. *See* Iowa Code § 86.26; *Quaker Oats Co. v. Ciha*, 552 N.W.2d 143, 149 (Iowa 1996). The court's statutory scope of review provides for reversal if the Agency's decision was "unreasonable, arbitrary, capricious, or an abuse of discretion." Iowa Code § 17A.19(10)(n). An agency's action is arbitrary or capricious when "it is taken without regard to the law or facts of the case" and unreasonable when "it is clearly against reason and evidence." *Soo Line R.R. v. Iowa Dep't of Transp.*, 521 N.W.2d 685, 688-89 (Iowa 1994). Unreasonableness is defined as action in the face of evidence that leaves "no room for difference of opinion among reasonable minds, or not based on substantial evidence." *Stephenson v. Furnas Elec. Co.*, 522 N.W.2d 828, 831 (Iowa 1994). "Abuse of discretion is synonymous with unreasonableness, and involves lack of rationality, focusing on whether the agency has made a decision clearly against reason and evidence." *Id.*

The Courts are bound by the Agency's findings of fact if supported by substantial evidence. *See Mosher v. Dep't of Inspections & Appeals*, 671 N.W.2d 501, 508 (Iowa 2003). The Court may reverse, modify, or grant other relief when agency action is based on fact determinations "not supported by substantial evidence in the record before the court when that record is viewed as a whole." Iowa Code § 17A.19(10)(f) (2001). "Substantial evidence" is statutorily defined as

the quantity and quality of evidence that would be deemed sufficient by a neutral, detached, and reasonable person, to establish the fact at issue when the consequences resulting from the establishment of that fact are understood to be serious and of great importance. *Id.* Iowa Code § 17A.19(10)(f)(1). "On appeal, our task 'is not to determine whether the evidence supports a different finding; rather, our task is to determine whether substantial evidence … supports the findings actually made." *Cedar Rapids Community School District v. Pease*, 807 N.W.2d 839, 845 (Iowa 2011).

A. THE COMMISSIONER COMMITTED AN ERROR OF LAW IN CONCLUDING THAT DEFENDANTS' OFFERING WOOFF A JOB THAT HE WAS UNQUALIFIED TO DO WAS SUFFICIENT TO AVOID AN INDUSTRIAL DISABILITY AWARD

> Defendants argue:
> Clearly the employer believed Claimant had the necessary qualifications to successfully complete the jobs based on their good faith offers. The nature of the job was within the claimant's expertise —construction work. It was not a job that was completely and totally different from the nature of the claimant's past work experience.
>
> (Def. Sup Ct Brief p. 27).

There is no proof that their offer was made in "good faith." In fact, emails between defense counsel and the employer make it clear that Wooff was not their first or only choice for the job (Cl. Ex. p. 299). Wooff was offered the job on August 25, 2021, with a start date of September 1, 2021 (Cl. Ex. 7 p. 294). Emails between defense counsel and the employer all appear to take place on August 27,

2021, state that they are interviewing "several candidates" and have been looking to fill the position for 4 weeks (Cl. Ex. 7 pp. 298-301). The emails continued to discuss the other interviews that had already been scheduled/conducted and confirming that Wooff declined the jobs (Cl. Ex. 7 pp. 298-301). They conclude with an email from defense counsel, "I think it was very important that we made the good faith offer of employment to him" (Cl. Ex. 7, p. 301). Wooff was released at MMI for his shoulder on August 10, 2021 (Jt. Ex. 7. p. 387). According to these emails, the employer was already over two weeks into the process of looking for someone to fill these positions, and yet they did not offer the jobs to Wooff until August 27, 2021 (Cl. Ex. 7, p. 298). It becomes quite clear that the offer to Wooff was for purposes of the defense of this workers' compensation claim and not a "good faith" offer.

The contention that "The nature of the job was within the claimant's expertise—construction work. It was not a job that was completely and totally different from the nature of the claimant's past work experience" is a stretch (Appellees' Sup Ct Brief, p. 27). While Wooff had performed many drywall duties, he did not have managerial, supervisory, or estimator experience. To say it's "not a job that was completely and totally different from the nature of claimant's past work experience" is saying it is a good faith offer if you offer a registered nurse a surgeon's job.

Defendants also argue:

> Claimant asserts that he was not at maximum medical improvement for his neck and later underwent surgery on his neck; therefore, he was not physically able to perform the jobs that were offered to him. Defendants offered the jobs to Claimants in good faith. Had Claimant accepted a job and then needed to undergo surgery in the future, the employer's typical policies regarding medical leave would have been followed. Claimant continued to work for Defendants throughout the course of his medical treatment related to his right shoulder. There is no reason to believe that Claimant could not have continued working in a different position for Defendants during the course of his treatment for his neck.

(Def. Sup Ct Brief, p. 27).

First, it must be noted that the neck injury was DENIED by the defendants (although ultimately found compensable). So, their assertion that they would have worked with Wooff if he had to miss work does not acknowledge that they considered the neck to be a personal condition and not part of the workers' compensation claim. Additionally, there is no evidence in the record that the employer would have followed "typical policies" or even what those "typical policies" are. Especially considering the emails made it clear the employer was intent on filling these positions and not waiting months for Wooff to recover from his work injury or go through training.

Again, the timeline is important. Wooff was released at MMI for his shoulder on August 10, 2021. However, during this appointment, he was referred to Dr. Andrea Pugely for his neck condition (Jt. Ex. 7, p. 398). He was offered the jobs on August 27, 2021, to begin September 1, 2021 (Cl. Ex. 7, pp. 294, 298). He

saw Dr. Pugely on August 31, 2021, at which time it was decided he would need surgery "C3-4 ACDF" (Jt. Ex. 7, p. 392). BEFORE Wooff would have even started the job, he was aware that he needed surgery to his neck.

The Commissioner erred as a matter of law in concluding that defendants' offer of work was in good faith such that it limited Wooff to only his functional impairment ratings.

B. SUBSTANTIAL EVIDENCE SUPPORTS THAT WOOFF'S MENTAL HEALTH INJURY SHOULD BE CONSIDERED IN THE INDUSTRIAL DISABILITY ANALYSIS

Defendants assert that the Commissioner specifically addressed Wood's 40% body as a whole impairment rating related to Wooff's mental health and rejected it (Appellees' Sup Ct Brief, p. 29).

The basis for the rejection is at issue. In the 03/21/2024 Arbitration Decision, the Deputy held:

> Based upon the evidence, I find that the claimant proved that his mental health issues were aggravated or lit up as a sequela of his right shoulder and neck injuries. Therefore, I find that his mental health issue arose out of, and in the course of his employment with the defendant. I also find that the claimant has shown this to be a cause of permanent disability based upon the foregoing reasons (03/21/24 Arb. Dec., p. 52).

However, the Deputy – inconsistently – concludes:

> Dr. Woods opined that the claimant's mental health issues caused a 40 percent functional loss. However, Dr. Woods opined that this may not be a permanent impairment. This casts doubt on the strength of Dr. Woods' opinions on this specific issue. (03/21/24 Arb. Dec., p. 55).

Dr. Woods did not say it may not be permanent. What Dr. Woods DID say regarding addressing permanency in mental health claims is the following:

> …Mr. Wooff's chronic pain is a big part of his ongoing Adjustment Disorder and its other related symptoms. If the pain is not resolved (or reduced to at least to a manageable level) then it is very likely (within a reasonable degree of medical certainty) that the Adjustment Disorder will be permanent. (Cl. Ex. 2, pp. 139-140).
>
> …
>
> Dr. Segal further opines:
>
> To begin… let me first state that the impairment rating given reflects how Mr. Wooff is doing at this moment in time. It is not necessarily permanent (and this MD is not able to assert that the client is at maximum medical improvement (MMI)), because his mental health treatment up to this point has not been optimal… Once the client has been receiving comprehensive mental health treatment for at least six months (possibly a year or more), only then can he truly be assessed for the possibility of MMI regarding his mental health ailments. For the sake of clarity, I will explain further below.
>
> In mental health, determining a date for maximum medical improvement (MMI) is extremely challenging due to its multifactorial nature. To put it simply, it is always possible one's mental health symptoms could improve with a new medication regimen or substantial lifestyle changes, but the question of how likely and how much change is problematic to predict. With that said, in this case, Mr. Wooff's chronic pain, increased weakness, and mobility issues are a key part of his Adjustment Disorder and these facets seem extremely unlikely to fully resolve. Given this fact, it is therefore improbable his mental health symptoms will fully resolve either, but with the caveat that, as said earlier, that it is always possible, given the complexity of the brain and the numerous psychosocial factors at play, that Mr. Wooff's mental health symptoms may ebb and flow over time. Therefore, if a specific date is needed for my impairment rating given in Question #9, it would be August 8, 2023, the date of this MD's examination of Mr. Wooff. If the client does not get further treatment

> for his mental health ailments, then it would be fair to call that his date of MMI.
>
> With all that said, this MD is highly doubtful that Mr. Wooff will be able to work a physical labor job ever again. Given his age and education level, it also seems improbable that he will be able to retrain for a nonphysical, primarily cerebral line of work either. For the sake of his quality of life and peace of mind, we must proceed forward under the assumption he is not able to work at all at this point in time. This MD is not comfortable, however, assigning permanent restrictions/ limitations and will defer to Dr. Alden, the VA mental health staff, and my physical medicine colleagues to make such decisions. (Cl. Ex. 2, pp. 140-141).

The status of Wooff's mental health condition AT THE TIME OF THE HEARING is what determines what the award should be. While Dr. Woods states Wooff "could" improve with treatment (he had not had any due to it being denied by Defendants), if such improvement were to occur, that would leave Defendants with a review-reopening option to reduce the award. Dr. Woods clearly finds that at the time of the hearing, Wooff has a 40% permanent impairment to his body as a whole as result of his mental health condition which the agency found arose out of and in the course of his employment. This 40% should be considered in the industrial disability analysis.

If it determined that the job offers limit Wooff to the functional impairment award only, the 40% should also be considered in that award.

C. THE COMMISSIONER ERRED AS A MATTER OF LAW WHEN IT IMPROPERLY USED AGENCY EXPERTISE IN DETERMINING WOOFF'S PERMANENT IMPAIRMENT

If it is determined that Wooff is limited to recovering under the functional impairment analysis, the Commissioner improperly used agency expertise in determining Wooff's permanent impairment.

Specifically, the Commissioner found Dr. Segal's rating for the distal clavicle excision to be "inconsistent with prior agency precedent" (10/18/24 App. Dec., p. 5) Wooff submits that the dissenting opinion in *Koeller v. Cardinal Logistics Mgmt. Corp.*, 2025 Iowa App. LEXIS 1008 is the correct result regarding the rating of the distal clavicle excision. Justice Langholz stated:

> Despite all this, the commissioner interpreted the AMA Guides to require application of the 25% modifier from table 16-18 to reduce the upper-extremity-impairment rating of section 16.7b and table 16-26. The commissioner gave no reasoning for this interpretation here because the deputy commissioner considered herself bound by agency precedent and the commissioner affirmed without additional reasoning. But in that prior case—also involving a resection arthroplasty of the distal clavicle like Koeller's—the commissioner cited the general provision at the start of section 16.7 that "[t]he severity of the impairment due to" the conditions covered in section 16.7 "is rated separately according to Tables 16-19 through 16-30 and then multiplied by the relative maximum value of the unit involved as specified in Table 16-18." *Jay v. Archer Skid Loader Serv., LLC*, No. 19003586.01, 2022 WL 17078713, at *7 (Iowa Workers' Comp. Comm'r Aug. 23, 2022) (quoting AMA Guides § 16.7, at 498). And then the commissioner summarily concluded that the AMA Guides "require application of a 25 percent multiplier. This results in a 2.5 percent impairment for a distal clavicle excision under the plaint text of the AMA Guides." The commissioner did not discuss the rest of the text and context of section 16.7.

If one focused on only the general provision at the start of section 16.7, it is understandable why one might think that the AMA Guides always requires application of the modifier. After all, table 16.26 does fall within "Tables 16-19 through 16-30." AMA Guides § 16.7, at 498. But it is error to read that general provision in isolation. "Perhaps no interpretive fault is more common than the failure to follow the whole-text canon, which calls on the judicial interpreter to consider the entire text, in view of its structure and of the physical and logical relation of its many parts." Antonin Scalia & Bryan A. Garner, Reading Law: The Interpretation of Legal Texts 167 (2012) [hereinafter, "Reading Law"]. And considering all of section 16.7 in context as I do above—rather than stopping with the general provision—the commissioner's interpretation becomes untenable.

To accept it, one must ignore that table 16.26 already gives an upper-extremity-impairment rating rather than a joint or digit rating that would need to be converted with the modifier. One must ignore the omission of the asterisk directing that the modifier be applied. One must ignore that the example given for table 16.26 does not apply the modifier. And one must ignore the consistent contrast on all these points between the provisions giving upper-extremity-impairment ratings and those provisions where the modifier is expressly directed to be applied.

To be sure, the general provision could have been written more clearly to explain how it interacts with table 16.26 and the others that give upper-extremity-impairment ratings and thus have no applicable relative value on table 16.18. But even if some tension remains between that general provision of section 16.7 on one hand and section 16.7b and table 26.26 on the other, section 16.7b and table 16.26 are the more specific provisions and thus must control over the more general provision. *See* Reading Law, at 183 (explaining that under the "general/specific canon" of textual interpretation, "[i]f there is a conflict between a general provision and specific provision, the specific provision prevails" and is "treated as an exception to the general rule"); *Est. of Kahn by Rowe v. City of Clermont*, 22 N.W.3d 252, 262 (Iowa 2025) (applying the canon to reverse the district court's interpretation that relied on a general statute rather than the specific).

Correctly interpreted, the AMA Guides thus does not apply a modifier from table 16-18 to reduce an upper-extremity-impairment rating given by section 16.7b and table 16-26. The commissioner's contrary interpretation is wrong. *Koeller v. Cardinal Logistics Mgmt. Corp.*, 2025 Iowa App. LEXIS 1008, *26-30

…

Koeller argues that the commissioner's decision to apply the modifier —despite Koeller's expert's interpretation of the AMA Guides that the modifier should not be applied—violated paragraph "x" for two reasons. First, he contends that the reliance on the commissioner's own interpretation of the AMA Guides in prior cases was impermissible use of "agency expertise." Iowa Code § 85.34(2)(x). Second, he contends that his expert's interpretation of the AMA Guides is the correct one and thus the commissioner's decision based on a wrong interpretation of the AMA Guides was not made "solely by utilizing" the AMA Guides. *Id.*

I am skeptical that Koeller's first argument could succeed. It is the commissioner's duty to make factual determinations on contested issues, including an employee's percentage of permanent impairment. *See Sherman v. Pella Corp.*, 576 N.W.2d 312, 322 (Iowa 1998), *abrogated on other grounds by* Iowa Code § 85.34(2)(x). So paragraph "x" is a mandate on the commissioner that such a determination shall be made "solely by utilizing" the AMA Guides. *Id.* § 85.34(2)(x); *see also Klein v. Whirlpool Corp.*, No. 1656402.03, 2024 WL 3696768, at *2 (Iowa Workers' Comp. Comm'r July 30, 2024). Complying with that mandate requires interpreting the AMA Guides to evaluate whether an expert's opinion on the percentage of impairment is based on a correct interpretation of the AMA Guides. *See id.* at *2-4; *Polk County v. Jones*, 74 Ark. App. 159, 164, 47 S.W.3d 904 (Ark. Ct. App. 2001) (rejecting an argument that the Arkansas workers' compensation commission could not make its own "finding of permanent impairment using the AMA Guides" rather than only relying on a physician's rating). Indeed, at times in Koeller's trial and appellate briefing and again at oral argument before our court, he has conceded that the commissioner could correct a clear error in an expert's interpretation of the AMA Guides. And we generally expect an agency to be consistent in its interpretations of governing

provisions. *See, e.g.*, Iowa Code § 17A.19(10)(h) (authorizing reversal of agency "[a]ction other than a rule that is inconsistent with the agency's prior practice or precedents, unless the agency has justified that inconsistency by stating credible reasons sufficient to indicate a fair and rational basis for the inconsistency"). So I fail to see how merely abiding by prior agency precedents on the interpretation of the AMA Guides is use of "agency expertise" rather than just following the law.

But we need not resolve this issue here. *Cf. Harrell v. Denver Findley & Sons, Inc.*, No. 21-0827, 986 N.W.2d 872, 2022 WL 2824746, at *2 (Iowa Ct. App. July 20, 2022) (leaving "for another case" the argument that "when a non-physician commissioner or deputy commissioner uses the [AMA Guides] to determine impairment, they necessarily rely on 'agency expertise'—a source forbidden by section 85.34(2)(x)" while instead reversing the commissioner's decision for violating the prohibition on use of lay testimony). Whatever the scope of the commissioner's authority to interpret the AMA Guidelines independently of the expert witnesses making assessments under the guidelines, the commissioner must interpret the AMA Guidelines correctly. Otherwise, the commissioner would not be "utilizing" the AMA Guides as required by paragraph "x"—he'd be using his own misinterpreted variation of the AMA Guides. And so I agree with Koeller's second argument that the commissioner violated paragraph "x" by using his erroneous interpretation of the AMA Guides here.

Still, that's not quite the end of the analysis. Judicial review of agency action is limited—and a creature of statute: Iowa Code section 17A.19. We must defer to the workers' compensation commissioner's factual findings, including his determination of the extent of Koeller's functional impairment, when supported by substantial evidence. *See* Iowa Code § 17A.19(10)(f), (11)(c); *Sherman*, 576 N.W.2d at 322. On the other hand, because the commissioner has not been delegated general authority to interpret Iowa Code chapter 85, we typically give no deference to his legal interpretations—reviewing for corrections of errors at law. *See Bridgestone Americas, Inc. v. Anderson*, 4 N.W.3d 676, 682 (Iowa 2024).

Neither party gives much attention in the briefing to how we should review the commissioner's interpretation of the AMA Guides. But

when pressed at oral argument, Koeller suggested it is a mixed question of law and fact—so we can correct textual interpretation errors—while his employer and its insurer advocated for treating it as a factual determination to which we must give deference. I interpret paragraph "x" and the corresponding administrative rule 876-2.4 to essentially incorporate the fifth edition of the AMA Guides into our workers' compensation law. *See Klein*, 2024 WL 3696768, at *2 (reasoning that the statute and administrative rule makes the AMA Guides "binding law" and that reading and applying its provisions "is akin to an application of the law to the facts of a case"); *Hill v. Am. Med. Response*, 2018 OK 57, 423 P.3d 1119, 1126-1135 (Okla. 2018) (reasoning that a similar statute "incorporated" the AMA Guides "into workers' compensation law" and rejecting many constitutional challenges); Iowa Code § 17A.6(2)-(3) (setting procedures for administrative rules that "adopt[] standards by reference to another publication"). And I see nothing in paragraph "x" that suggests a clear delegation to the commissioner of discretion to interpret the AMA Guides—quite the opposite, making the AMA Guides mandatory and prohibiting use of agency expertise is a restriction on the commissioner's authority.

I would thus treat the commissioner's interpretation of the meaning of the AMA Guides—like the one here—as a question of law that we review for corrections of errors at law without giving any deference to the commissioner. *See* Iowa Code § 17A.19(10)(c) (authorizing reversal of agency action "[b]ased upon an erroneous interpretation of a provision of law whose interpretation has not clearly been vested by a provision of law in the discretion of the agency); *id.* § 17A.19(11)(b) (directing that the court "[s]hould not give any deference to the view of the agency with respect to particular matters that have not been vested by a provision of law in the discretion of the agency"); *Bridgestone*, 4 N.W.3d at 682; *see also Klein*, 2024 WL 3696768, at *2 ("It would be legally erroneous to ignore the provisions of the AMA Guides, Fifth Edition, when reviewing this case."). Such an approach mirrors the general rule that the interpretations of texts are matters of law for the court to decide—not the factfinder—subject to appellate review for corrections of errors at law. *See Hartig Drug Co. v. Hartig*, 602 N.W.2d 794, 797 (Iowa 1999) (reviewing "the construction and interpretation of a contract as a matter of law" when based on the text and context rather than extrinsic evidence).

> Of course, many disputes about the proper impairment rating under the AMA Guides will be about not the meaning of its provisions but rather the factual assessment of the claimant's condition by medical experts. Such disputes are still subject to substantial-evidence review —same as before the enactment of paragraph "x." *See* Iowa Code § 17A.19(10)(f). But the dispute here is a purely legal question of textual interpretation. So it is proper for us to correct any error in that interpretation on judicial review.
>
> Bottom line, the workers' compensation commission erroneously interpreted the AMA Guidelines to require use of a modifier from table 16-18 to reduce an upper-extremity-impairment rating given by table 16-26. Koeller is entitled to reversal of that erroneous interpretation and his resulting impairment rating and permanent partial disability award. I would thus reverse the district court and the workers' compensation commissioner on Koeller's appeal and remand for the commissioner to make a new permanent impairment rating consistent with an accurate interpretation of the AMA Guides. As the majority holds otherwise, I respectfully dissent from the part of its opinion addressing Koeller's appeal. But I too see no merit in the cross-appeal by Koeller's employer and its insurer. So I concur in the rest of the opinion addressing that cross-appeal. *Koeller v. Cardinal Logistics Mgmt. Corp.*, 2025 Iowa App. LEXIS 1008, *31-37.

IF it is determined that Wooff's recovery is limited to functional impairment, he should be awarded either Dr. Segals 13% BAW OR Dr. Nepola's 10% BAW for the shoulder part of the injury in addition to Dr. Segal's 35% BAW for the cervical spine, and Dr. Woods's 40% BAW for the mental health injury(Cl. Ex. 1, pp. 58, 67; Cl. Ex. 2, p. 140; Jt. Ex. 7, p. 387).

CONCLUSION

Wherefore, Wooff requests the Court find that the offer of work made by Defendants was not suitable and remand the matter back to the Agency with

instructions to use the industrial disability analysis and for such analysis to include consideration of Dr. Nepola and Dr. Segal's distal clavicle excision ratings and Dr. Woods's permanent impairment rating for Wooff's mental health injury. In the alternative, if it is found that the basis of Wooff's compensation is his functional impairment rating, then the matter should be remanded back with instructions to utilize the appropriate impairment rating for the distal clavicle excision, Dr. Segal's 35% BAW for the cervical spine, and the mental health rating by Dr. Woods.

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CERTIFICATE OF FILING

I, Denise M. Wall, an employee of Wertz Law Firm, attorneys for the Appellant, certify the attached Reply Brief was filed with the clerk of the Iowa Supreme Court via electronic filing on this 18th day of December 2025.

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CERTIFICATE OF SERVICE

I, Denise M. Wall, an employee of Wertz Law Firm, attorneys for the Appellant, certify the attached Reply Brief filed with the Clerk of the Iowa Supreme Court was forwarded to all counsel via the electronic filing system and email on this 18th day of December, 2025, and by U.S. Mail for any party not registered to receive notice of filings via the electronic filing process.

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